NCE EXAM PREP

Practice Exam 23

Domain 5

Indirect Client Care

21 Practice Questions | 28 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 04. Use the suggested 28-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

Which information best shows whether care coordination is helping a client?

A) The number of meetings held and the number of professionals who attended

B) Progress on client-defined outcomes, resolution of access barriers, follow-through, and the client's experience of the process

C) Stakeholder satisfaction with the counselor even when the client's identified barriers remain unchanged

Question 2

Which referral documentation most supports later continuity?

A) The identified need and preferences, resource offered, client response, and planned follow-up

B) A note that says only "referral made" without naming the purpose or next step

C) A copy of the entire resource directory without indicating which option was discussed

Question 3

> A hard-of-hearing client uses spoken English and requests real-time captioning for a counseling workshop. The clinic offers an American Sign Language interpreter, but the client does not use ASL and says captioning is the effective access method.

What should the counselor arrange?

A) The ASL interpreter because interpretation is already available through the clinic

B) A front-row seat so the client can rely on lipreading during the workshop

C) Qualified real-time captioning consistent with the client's stated communication method

Question 4

> During an intake, Rosa first says her sleep difficulty began "sometime in the spring" and later says it may have started after Memorial Day. She is unsure of the exact date, and no other record establishes it.

How should the counselor document symptom onset?

A) Select Memorial Day as the onset date because it is the most specific date mentioned

B) Record Rosa's approximate time frame and uncertainty rather than converting it into an exact date

C) Describe Rosa as an unreliable historian because the two statements are not identical

Question 5

> Tomas wants a neuropsychological evaluation. The counselor verifies that Tomas's insurance requires a referral from his primary care provider before an in-network evaluator can schedule him. Tomas has a primary care provider but did not know about the requirement.

What referral support is most appropriate?

A) Diagnose the suspected condition so the evaluator can bypass the insurance requirement

B) Tell Tomas that the evaluation is unavailable because the counselor cannot issue the required referral

C) Explain the verified prerequisite and help Tomas request the referral from his primary care provider

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Question 6

> Between sessions, Jae sends several nonurgent portal messages describing brief interactions at work. The messages contain no safety concern and touch on three recurring themes related to the current counseling goal. The clinic's stated practice is to address nonurgent clinical material during appointments.

How should the counselor prepare for the next session?

A) Create a new treatment plan from the messages before asking Jae which material matters most

B) Plan to review every message in chronological order even if doing so uses the entire session

C) Organize the recurring themes and relevant questions, then invite Jae to prioritize what to explore

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Question 7

> Solange asks her counselor for help locating an immigration legal consultation before a filing deadline. She prefers services in Haitian Creole. The counselor's directory lists a nonprofit clinic, but its current eligibility rules, language capacity, and availability before the deadline are unclear.

What is the counselor's best next step?

A) Verify the clinic's eligibility, language access, and deadline capacity, then offer Solange help connecting if it is a fit

B) Review Solange's immigration forms and advise her which legal claim is most likely to succeed

C) Give Solange the unverified clinic listing and assume the organization will arrange any needed language support

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Question 8

After a manic episode, Serena asks her sister to help with budgeting and authorizes care-planning communication. Serena defines progress as paying bills on time while retaining some discretionary spending. Her sister says any discretionary purchase shows that Serena is unstable.

How should the counselor structure their collaboration?

A) Help Serena and her sister use client-approved, observable indicators and clarify the sister's support role

B) Adopt the sister's standard because restricting all discretionary spending offers the greatest protection

C) End the sister's participation because disagreement makes family support clinically inappropriate

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Question 9

Malik has authorized his counselor and physical therapist to coordinate care for chronic pain. The therapist reports that Malik avoids a prescribed activity progression. Malik tells the counselor that his pain sharply increased after the progression was accelerated and that he still wants to become more active.

What should the counselor facilitate?

A) Tell the therapist that Malik is nonadherent and needs stronger accountability

B) Encourage Malik to follow the progression without question because the therapist designed it

C) Review the pacing change and Malik's response with both parties and help align the plan with his activity goal

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Question 10

After the counselor demonstrates a grounding exercise, the client accurately repeats the steps and then says, "I am not sure I could do this at work when people are watching." The counselor is documenting the client's response.

Which note entry is most accurate?

A) The client mastered the grounding exercise and committed to using it daily at work.

B) The client rejected the grounding exercise because of poor motivation.

C) The client accurately described the steps and expressed uncertainty about using the exercise around coworkers.

Question 11

Priyanka tells her counselor that her prescriber ordered routine laboratory monitoring for a mood-stabilizing medication, but the laboratory is open only during her work hours. She is considering skipping the test. She reports no symptoms and authorizes coordination with the prescriber.

What should the counselor do?

A) Tell Priyanka that routine monitoring can be postponed when she feels well

B) Recommend stopping the medication until she can attend the laboratory

C) Document the scheduling barrier and help Priyanka and the prescriber identify an approved, accessible monitoring option

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Question 12

At a planned final session, Nina reports that she now gives required work presentations instead of avoiding them. She still expects some anxiety before major presentations and identifies strategies and supports she plans to continue using.

What should the closing note emphasize?

A) That Nina is cured because she no longer avoids required presentations

B) The original goal, relevant interventions, functional progress, remaining vulnerability, and maintenance or follow-up plan

C) A verbatim copy of every prior progress note so the entire episode of care appears in one entry

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Question 13

DeShawn is considering a paid training program while receiving disability benefits. He has authorized coordination with a vocational counselor and a training coach, who give conflicting statements about how earnings may affect benefits. Neither professional is a qualified benefits specialist.

What should the counselor facilitate?

A) Calculate the likely benefit change and advise DeShawn whether to accept the training position

B) Arrange clarification with a qualified benefits specialist and help the team incorporate verified information into DeShawn's decision

C) Recommend that DeShawn decline all paid work so his current benefits cannot be affected

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Question 14

> Noor cannot attend a housing-team meeting because of work but has authorized the counselor, a housing navigator, and a benefits counselor to meet. Noor previously identified remaining near her child's school and avoiding a shared apartment as priorities. During the meeting, the team finds an immediately available shared unit outside the school area.

How should the counselor handle the new option?

A) Approve the unit for Noor because immediate availability outweighs the earlier preferences

B) Record the option without committing Noor and arrange for her to review it before the team changes the plan

C) Reject the unit permanently on Noor's behalf because it differs from the priorities she previously stated

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Question 15

Which progress-note practice most clearly communicates the counselor's clinical reasoning without overstating it?

A) Record a chronological account of everything said while omitting an assessment and next plan

B) Repeat the client's diagnosis at each visit without linking it to current session information

C) Connect relevant reported and observed data to a supported assessment and a plan tied to the current goal

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Question 16

> Morgan previously authorized a workplace wellness coach to receive limited counseling updates. Morgan now states that the coach should no longer be involved and instead wants future coordination with a union representative. Clinic policy permits clients to revoke authorization at any time, with revocation applying to future disclosures.

What should the counselor do?

A) Stop future updates to the coach, document the change, and coordinate with the union representative after Morgan authorizes it

B) Send the coach a final clinical summary before ending communication because the prior authorization remains in the record

C) Continue the existing arrangement until the coach agrees that the transition is appropriate

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Question 17

> Evelyn relies on her sister for grocery trips and rides to appointments. The sister will be away for two months. Evelyn has authorized temporary coordination with a cousin and a senior-center volunteer, and she wants to remain in charge of which tasks each person handles.

What should the counselor help the group create?

A) A temporary coverage plan with Evelyn-selected responsibilities, confirmation steps, and a backup for missed support

B) An agreement that the cousin will assume every task because family help is more dependable than volunteer help

C) A suspension of Evelyn's appointments until her sister returns and resumes the usual routine

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Question 18

> Harper tells the counselor that she feels unheard by her psychiatric prescriber and wants a second opinion about her medication. She reports no acute symptoms and does not want to stop the medication on her own. Her insurance plan lists several psychiatric prescribers, but the counselor has not verified which are accepting new patients.

What should the counselor do?

A) Defend the current prescriber's plan so Harper does not disrupt treatment

B) Verify appropriate second-opinion options, review them with Harper, and support records coordination with her authorization

C) Recommend that Harper stop the medication before the new evaluation so the second opinion is unbiased

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Question 19

How should a referral directory distinguish peer-led support from licensed clinical services?

A) List both under the same service label so clients are not influenced by professional titles

B) Identify provider type, service scope, and relevant qualifications so clients can understand what each resource offers

C) Exclude peer-led programs because only licensed services belong in a counseling referral directory

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Question 20

> Luis lives in transitional housing where residents may not store medication in the shared refrigerator. He shows his counselor a pharmacy label stating that his medication requires refrigeration. He has one dose due that evening, reports no symptoms, and authorizes coordination with the pharmacy, prescriber, and housing case manager.

What should the counselor do?

A) Promptly coordinate with the pharmacy and prescriber about verified storage requirements and, with Luis, address a safe housing arrangement

B) Advise Luis to keep the medication unrefrigerated in a locked bag until his housing situation changes

C) Tell Luis to stop using the medication until the residence changes its refrigerator policy

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Question 21

> A session ends with two specific agreements: the client will call a tenant-support service by Friday, and the counselor will send an updated food-resource list the same day. The draft note's plan section says only, "Follow up on housing and food."

How should the counselor revise the plan?

A) Record each agreed action, who will complete it, and the applicable time frame

B) Keep the general wording so the counselor and client can decide responsibilities later

C) Remove the practical tasks because a clinical plan should include symptoms only

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PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: B

Correct Answer: Progress on client-defined outcomes, resolution of access barriers, follow-through, and the client's experience of the process

Explanation: Coordination is useful when it advances outcomes and reduces barriers without creating excessive burden for the client. Meeting counts and stakeholder satisfaction are process measures, not evidence that the client is benefiting. The client's experience is therefore part of evaluating effectiveness.

Question 2 — Correct: A

Correct Answer: The identified need and preferences, resource offered, client response, and planned follow-up

Explanation: These elements show why the referral was made, how it fit the client, and what should happen next. A vague statement cannot guide later follow-through. Copying a whole directory does not identify the option actually discussed.

Question 3 — Correct: C

Correct Answer: Qualified real-time captioning consistent with the client's stated communication method

Explanation: The client has identified captioning as the effective way to access spoken English. ASL interpretation does not meet that need when the client does not use ASL, and lipreading is not an equivalent replacement. Matching the support to the client's communication method enables meaningful participation.

Question 4 — Correct: B

Correct Answer: Record Rosa's approximate time frame and uncertainty rather than converting it into an exact date

Explanation: The note should preserve the level of precision Rosa actually provided. Choosing a specific date would create unsupported certainty, while labeling her unreliable adds a judgment not established by ordinary recall uncertainty. An approximate range remains clinically usable.

Question 5 — Correct: C

Correct Answer: Explain the verified prerequisite and help Tomas request the referral from his primary care provider

Explanation: The counselor has identified a concrete procedural step and can help Tomas navigate it. The requirement does not make the evaluation unavailable; it identifies the professional who must initiate the insurance pathway. The counselor should not create a diagnosis to bypass that process.

Question 6 — Correct: C

Correct Answer: Organize the recurring themes and relevant questions, then invite Jae to prioritize what to explore

Explanation: The messages can inform efficient preparation without allowing the counselor to determine their meaning or priority alone. Inviting Jae to choose the focus preserves collaboration and the clinic's stated use of sessions for nonurgent material. Chronological review of every message may displace the work Jae finds most important.

Question 7 — Correct: A

Correct Answer: Verify the clinic's eligibility, language access, and deadline capacity, then offer Solange help connecting if it is a fit

Explanation: Verification addresses the factors that determine whether the legal resource is usable before the deadline. The counselor can support access without interpreting legal claims or forms. An unverified listing could consume time Solange does not have.

Question 8 — Correct: A

Correct Answer: Help Serena and her sister use client-approved, observable indicators and clarify the sister's support role

Explanation: Observable indicators such as timely bills can turn a vague disagreement into a collaborative plan grounded in Serena's goal. The sister's support can remain useful without giving her unilateral control over Serena's finances. Disagreement alone does not require excluding an authorized support person.

Question 9 — Correct: C

Correct Answer: Review the pacing change and Malik's response with both parties and help align the plan with his activity goal

Explanation: Coordination can bring the reported pain flare and the therapist's progression plan into the same discussion. A shared review supports consistent pacing while retaining Malik's goal of greater activity. Labeling him nonadherent would ignore the clinically relevant context.

Question 10 — Correct: C

Correct Answer: The client accurately described the steps and expressed uncertainty about using the exercise around coworkers.

Explanation: This entry records both demonstrated recall and the client's stated concern about real-world use. Mastery and daily commitment were not established. Describing uncertainty as rejection would also distort the client's response.

Question 11 — Correct: C

Correct Answer: Document the scheduling barrier and help Priyanka and the prescriber identify an approved, accessible monitoring option

Explanation: The counselor can support access to the monitoring ordered by the prescriber without deciding whether it is medically necessary. Coordination may identify different hours, an approved location, or another prescriber-directed solution. Feeling well does not authorize the counselor to waive monitoring or stop medication.

Question 12 — Correct: B

Correct Answer: The original goal, relevant interventions, functional progress, remaining vulnerability, and maintenance or follow-up plan

Explanation: A closing note should summarize meaningful change while representing the client's continuing anxiety accurately. The maintenance plan supports continuity if symptoms recur or additional care is needed. Calling Nina cured would overstate the evidence, and copying all prior notes would obscure the episode's clinical summary.

Question 13 — Correct: B

Correct Answer: Arrange clarification with a qualified benefits specialist and help the team incorporate verified information into DeShawn's decision

Explanation: The conflicting advice concerns specialized benefit rules outside the identified stakeholders' expertise. Verified information from a qualified source allows DeShawn to make an informed choice without the counselor calculating benefits or discouraging work categorically. Coordination should resolve the information gap rather than substitute guesswork.

Question 14 — Correct: B

Correct Answer: Record the option without committing Noor and arrange for her to review it before the team changes the plan

Explanation: The meeting can gather useful information, but the new option conflicts with priorities Noor previously stated. Presenting it to Noor preserves her authority to weigh urgency against location and living arrangement. The team should neither accept nor permanently reject the unit on her behalf.

Question 15 — Correct: C

Correct Answer: Connect relevant reported and observed data to a supported assessment and a plan tied to the current goal

Explanation: This structure makes the counselor's reasoning visible while grounding it in information from the session. A diagnosis alone does not explain the current assessment or next action. An exhaustive chronology can still fail to communicate clinical meaning.

Question 16 — Correct: A

Correct Answer: Stop future updates to the coach, document the change, and coordinate with the union representative after Morgan authorizes it

Explanation: The stated policy makes Morgan's revocation effective for future disclosures. The coordination plan should therefore remove the coach and add the union representative only after the new authorization is in place. A final clinical disclosure to the coach would contradict Morgan's current direction.

Question 17 — Correct: A

Correct Answer: A temporary coverage plan with Evelyn-selected responsibilities, confirmation steps, and a backup for missed support

Explanation: The plan addresses a predictable gap while preserving Evelyn's control over how help is divided. Confirmation and backup steps reduce the chance that an essential task will be missed. Assigning everything to one person would disregard Evelyn's stated preference and create a single point of failure.

Question 18 — Correct: B

Correct Answer: Verify appropriate second-opinion options, review them with Harper, and support records coordination with her authorization

Explanation: Harper can seek another qualified opinion without the counselor endorsing or opposing a medication regimen. Verifying availability prevents the insurance list from functioning as an untested referral. Stopping medication before the evaluation would be medication advice outside the counselor's role.

Question 19 — Correct: B

Correct Answer: Identify provider type, service scope, and relevant qualifications so clients can understand what each resource offers

Explanation: Accurate labeling allows clients and counselors to distinguish complementary peer support from clinical assessment or treatment. Peer-led resources can be valuable without being represented as licensed care. Combining the labels would obscure meaningful differences in role and scope.

Question 20 — Correct: A

Correct Answer: Promptly coordinate with the pharmacy and prescriber about verified storage requirements and, with Luis, address a safe housing arrangement

Explanation: The label and imminent dose create a time-sensitive access problem that should be clarified by the medication professionals. Luis and the housing case manager can then address a verified storage solution. The counselor should not advise altered storage or discontinuation without appropriate medication guidance.

Question 21 — Correct: A

Correct Answer: Record each agreed action, who will complete it, and the applicable time frame

Explanation: A specific plan supports accountability and allows the next session to assess follow-through accurately. The draft's general wording loses the distinct responsibilities and deadlines already agreed upon. Practical actions belong in the plan when they support the counseling goals.
